

NOVO NORDISK A/S

THE CALL – SEPTEMBER 2024 BUY at DKK 912 Target DKK 1,011 (FCFF DCF) +10.9% upside	THE OUTCOME – JULY 2026 ≈ DKK 330 -64% since the pitch Peaked at DKK 1,033 in June 2024, then de-rated
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[Accompanying Excel Model: Click here to access the full financial model](#)

This retrospective reviews the September 2024 investment thesis on Novo Nordisk against subsequent business performance and share price outcomes. It compares the original valuation assumptions with realised financial results, assesses which aspects of the original thesis were supported by subsequent outcomes and which did not, and discusses how the valuation framework would be revised based on information available today. The accompanying Excel model has been rebuilt using an FCFF framework with an updated sensitivity analysis and forecast-versus-actual comparison.

1. Original thesis and valuation framework (September 2024)

The recommendation was BUY with a DCF-derived target of DKK 1,011 per share against a price of DKK 912 – implied upside of 10.9%. The valuation was a six-year FCFF model discounted at a WACC of 4.31% (CAPM cost of equity of 4.29%, built from a Danish 10-year risk-free rate of 2.2%, a beta of 0.88 versus the OMXC25 and a 2.4% market risk premium) with terminal growth of 1.9%. Revenue and capex assumptions followed sell-side consensus: growth of 24% into 2025, fading to 5% by 2029, on a broadly held around 47% EBITDA margin.

The qualitative case rested on four pillars:

- Novo held a 54.8% value share of the GLP-1 segment, with third-party forecasts of a ~33% market CAGR through 2032. Ozempic and Wegovy were driving ~25% group sales growth, and North America alone contributed 80% of H1 2024 growth. GLP-1 leadership in a hyper-growth market.
- Wegovy's cardiovascular (SELECT/MACE) indication, Ozempic's CV risk-reduction label, and a semaglutide pipeline spanning MASH, Alzheimer's, HFpEF, CKD and PAD were expected to keep widening the addressable market. Label expansions compounding the moat.
- CagriSema's Phase 3 programme was framed as potentially best-in-class (15.6% weight loss in earlier data), with Awiqli positioned as the first once-weekly basal insulin. Next-generation pipeline optionality.
- Acquisitions (Forma, Inversago, Cardior) plus a step-up in capex to ~DKK 45bn signalled a widening franchise beyond diabetes and obesity care. Diversification and capacity.

The risks section of the original deck, to its credit, named the right categories: dependency on the diabetes and obesity segment, patent expirations, and intense competition. What it did not do was price them.

2. Realised outcomes (2024-2026)

The shares reached an all-time high of DKK 1,033 in late June 2024, exceeding the price target shortly before initiation. Performance thereafter was driven by a sequence of negative catalysts:

- **December 2024** – CagriSema Phase 3 (REDEFINE-1). Weight loss of 22.7% against market expectations of approximately 25%. The shares declined roughly 20% in a single session, removing the pipeline premium embedded in the valuation.

- **H1 2025** – US prescription momentum deteriorated. Wegovy and Ozempic script growth slowed as compounded semaglutide persisted in the US market and Eli Lilly's Zepbound gained share; the company also suffered a setback in Wegovy-related patent litigation.
- **May-August 2025** – guidance reductions and management change. FY2025 guidance was reduced twice, from 16-24% CER sales growth (February) to 13-21% (May) and 8-14% (July). CEO Lars Fruergaard Jørgensen departed in May; Mike Doustdar was appointed in August and announced approximately 9,000 headcount reductions (~11% of the workforce).
- **February 2026** – structural reset in guidance. FY2025 closed at DKK 309.1bn of sales (+10% CER, +6% reported) and DKK 127.7bn of operating profit, with gross margin declining from 84.7% to 81.0%. FY2026 guidance calls for adjusted sales and operating profit to decline 5-13% at CER, reflecting US net-pricing pressure, semaglutide loss of exclusivity in international markets, and competitive dynamics. The shares fell a further ~15% on the release.
- **2026 year-to-date** – stabilisation measures. Oral Wegovy launched in the US; the CagriSema NDA was filed in December 2025 with an FDA decision expected in Q4 2026; a DKK 15bn buyback is in progress. At approximately DKK 330, the shares stand 64% below the initiation price.

Forecast vs. actual

FY2024 (DKK m)	Model	Actual	Variance
Revenue	287,792	290,403	-0.9%
Operating profit	120,449	128,300	-6.1%
Net profit	98,717	100,988	-2.3%
Diluted EPS (DKK)	22.17	22.63	-2.0%

FY2025 (DKK m)	Model	Actual	Variance
Revenue	356,574	309,064	+15.4%
Operating profit	152,276	127,658	+19.3%
Net profit	124,637	102,434	+21.7%
Diluted EPS (DKK)	28.00	23.03	+21.6%

Variance shown as model over/(under) actual. FY2024 was close on every line; FY2025 was materially over-forecast on all of them.

3. Elements of the framework that performed

- Near-term estimation: The FY2024 forecasts landed within 1-6% on every line – revenue within 0.9%, EPS within 2%. The three-statement engine, the cost ratios and the working-capital build were sound. The shortfall in the recommendation is not attributable to modelling mechanics.
- Risk identification: The original SWOT and Porter's analysis explicitly flagged concentration in one franchise, patent expiry, and medium-high substitution risk. Every one of those materialised within eighteen months. The deficiency was the absence of these risks from the numerical assumptions, not their identification.
- Peer comparison: The comparable company analysis implied values far below the market price on EV/EBITDA and EV/Revenue. At the time I dismissed this as the peers failing to reflect Novo's growth. In hindsight the comps were carrying information: the market was pricing Novo as if hyper-growth were permanent, and the peer set did not support this.

- Margin of Safety assessment: A 10.9% upside is a thin margin of safety, and the memo said so. This was a momentum-supported BUY, not a conviction one.

4. Assumption review and forecast attribution

- Revenue trajectory: peak-cycle growth extrapolated. The largest single source of error. Adopting consensus 23% growth into 2025 required supply availability, US net pricing and competitive position to hold at peak levels simultaneously. Actual FY2025 growth was 10% at CER. Given the volume of capital entering the category, (Lilly's capacity expansion, compounders, oral small molecules in development) mean reversion in growth warranted treatment as the base case rather than a downside scenario.
- Cost of capital understated. The 4.3% WACC was constructed from a Danish risk-free rate and a beta measured against a domestic index in which Novo Nordisk is the dominant constituent. For a global pharmaceutical business generating the majority of its economics in US dollars and carrying binary clinical-trial risk, this materially understates the appropriate discount rate. With terminal growth of 1.9%, the WACC-minus-g spread was 2.4 points and approximately 90% of enterprise value sat in the terminal value. The model's sensitivity table shows a 50bp move in WACC shifting the price target by roughly 20%: the valuation outcome was primarily a function of the discount-rate assumption.
- Binary pipeline events not probability-weighted. CagriSema was reflected qualitatively rather than as an explicit scenario. A single Phase 3 readout removed ~20% of the equity value in one session, indicating the proportion of the market price contingent on best-in-class data. A probability-weighted valuation tree across the key readouts would have quantified this exposure at initiation.
- Pricing and exclusivity assumptions static. EBITDA margin was held at ~47% throughout the forecast period. US gross-to-net erosion, formulary dynamics, 340B effects and international semaglutide loss of exclusivity, the factors now driving negative FY2026 guidance, were not modelled. Margin durability was treated as a company attribute rather than an output of competitive and payer dynamics.

5. Methodology revisions

Applied to a current-day valuation of the same asset, the following revisions to the framework would be warranted:

- Cost of capital derived from the asset's economics. A global equity risk premium, a USD-weighted risk-free rate, and an unlevered beta drawn from a global pharmaceutical peer set place WACC in the 7-8% range. On the September 2024 inputs, this adjustment alone reduces the price target by more than one third and moves the implied rating from BUY to HOLD at DKK 912.
- Explicit pricing and exclusivity schedules. Gross-to-net erosion, loss-of-exclusivity step-downs by geography, and per-competitor share-loss assumptions modelled as discrete lines, rather than a single revenue growth vector.
- Probability-weighted pipeline valuation. Base business and pipeline valued separately, with probabilities and dates attached to the material readouts; the price target derived as the weighted sum. The sensitivity table functions as a decision input rather than an appendix.
- Terminal-value concentration limit. Where terminal value exceeds ~70% of enterprise value, the explicit forecast period should be extended or terminal growth reduced until the valuation is substantiated by forecastable years.

- Triangulation with relative valuation. Material divergence between intrinsic and relative valuation treated as a finding requiring reconciliation, rather than resolved by discounting the comparable set.

6. Conclusions

The recommendation's shortfall is attributable to assumption selection rather than model execution. Near-term estimates were accurate; the valuation was dominated by two inputs – the growth fade and the discount rate – that received the least scrutiny relative to their contribution to the price target. At DKK 912, the market price required peak growth rates and peak margins to persist across the forecast horizon and into the terminal period. When the CagriSema readout, US pricing developments and the FY2026 guidance reset repriced those expectations, the terminal value compressed accordingly.

The principal conclusions for the research process: discount-rate construction warrants the same scrutiny as the cash-flow forecast; peak-cycle consensus should be treated as a scenario rather than a base case; risks identified in qualitative analysis must be carried into the numerical assumptions; and persistent divergence between relative and intrinsic valuation should be reconciled, not dismissed. A DCF specifies the expectations embedded in a market price; the September 2024 target reflected expectations that subsequent fundamentals did not support.

Sources: Novo Nordisk FY2023-FY2025 annual reports and interim statements; company guidance announcements (Feb/May/Jul 2025, Feb 2026); original September 2024 pitch deck and model. Prices in DKK, Copenhagen listing (NOVO-B). Prepared as a personal learning retrospective – not investment advice. Full Excel model available upon request. **AI disclosure:** Generative AI tools were used to assist with drafting, editing and improving the clarity of this report. All financial modelling, valuation assumptions, analysis and conclusions are the author's own.